

D028

Advanced Health Assessment for Patients and Populations

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

Method © 2026 Erik Daniel Lindquist — Course Mastery Coach, armored drill. Items drafted with AI assistance under his direction.

How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

A 42-year-old accountant who recently adopted a dog reports sharp chest pain that is worse when she breathes in. Her temperature is 38.4 °C and a friction rub is heard. She has a family history of migraines. Which finding is subjective?

- A. Temperature of 38.4 °C.
- B. Friction rub on auscultation.
- C. Family history of migraines, because history is objective.
- D. Sharp chest pain worse on inspiration.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

In a note, which item belongs in objective data?

- A. Blood pressure of 148/92 mmHg.
- B. "I feel dizzy when I stand up."
- C. Patient reports nausea for two days.
- D. Patient's rating of pain as 6/10.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A new patient, a retired pilot who speaks three languages, arrives at a clinic with a newly remodeled waiting room for a visit about fatigue. What is the best first question?

- A. "You're not depressed, are you?"
- B. "Tell me about the tiredness you've been having."
- C. "Did flying cause your fatigue?"
- D. "Are you sleeping eight hours a night?"

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4.

A 30-year-old graduate student with two cats reports a headache. The clinician learns where it is and rates its severity at 7/10, then moves on to the physical exam. Her blood pressure is 118/76. What is the most important gap?

- A. Her blood pressure should be rechecked.
- B. Severity should always be asked last.
- C. Onset, duration, character, aggravating/relieving factors, radiation, and timing were not asked.
- D. Pet allergies should be explored first.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

A new rapid test is studied in a large urban hospital. Of 100 patients who have the disease, 85 test positive. Of 400 who do not, 380 test negative. The test costs \$12 and takes 15 minutes. What is its sensitivity?

- A. 95%
- B. 17%
- C. 85%
- D. 15%

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6. [TRANSFER — reversed stem / different variable]

Using the same study (85 of 100 with disease test positive; 380 of 400 without disease test negative), what is the specificity?

- A. 95%
- B. 76%
- C. 85%
- D. 5%

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

At a community health fair sponsored by a local bank, a nurse checks blood pressure for adults who have never been diagnosed with hypertension. Free water bottles are handed out. What level of prevention is this?

- A. Tertiary prevention.
- B. Health promotion only, since no disease is known.
- C. Primary prevention.
- D. Secondary prevention.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8. [TRANSFER — reversed stem / different variable]

A patient recovering from a stroke begins physical therapy to regain walking. What level of prevention is this?

- A. Primary prevention.
- B. Tertiary prevention.
- C. No prevention level applies after disease occurs.
- D. Secondary prevention.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

A nurse assessing a rural county drives through its neighborhoods noting housing, grocery stores, and parks. She also downloads the county's census data. The county fair is held every August. Which source is primary data?

- A. The county fair schedule.
- B. The census data.
- C. Both are primary because she is using them.
- D. The windshield survey she conducted.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10. [TRANSFER — reversed stem / different variable]

Which source is secondary data for a community assessment?

- A. State health department reports of diabetes rates.
- B. A survey the nurse designs and mails.
- C. Focus groups the nurse leads with residents.
- D. The nurse's observations at a bus stop.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

A Spanish-speaking patient arrives with her bilingual 12-year-old son, a straight-A student. She has abdominal pain and questions about sexual history are needed. The clinic has a phone interpreter service. What is the best approach?

- A.** Assume her beliefs based on her background.
- B.** Use the professional interpreter service.
- C.** Skip the sexual history to avoid discomfort.
- D.** Use the son, since he is bilingual and bright.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12.

A 58-year-old man with a history of heartburn who coaches youth soccer reports upper abdominal burning with sweating and shortness of breath during exertion. What should be ruled out first?

- A.** Muscle strain from coaching.
- B.** Gastroesophageal reflux, since he has a history of heartburn.
- C.** Acute coronary syndrome.
- D.** Anxiety, since symptoms are always worse with stress.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer D Concept 1

A 42-year-old accountant ~~who recently adopted a dog~~ reports sharp chest pain that is worse when she breathes in. Her temperature is 38.4 °C and a friction rub is heard. ~~She has a family history of migraines.~~ Which finding is subjective?

Target word: pick the subjective finding **Decides:** What the patient reports.

A	Inverted operation	Measured: objective.
B	Confusion set	Heard by the clinician: objective.
C	Camouflage	History is subjective, and this detail does not decide the item.
D	Correct	Pain is reported by the patient.

2. Answer A Concept 1 • transfer

In a note, which item belongs in objective data?

Target word: identify objective **Decides:** Observed or measured.

A	Correct	A measurement is objective.
B	Inverted operation	Patient report: subjective.
C	Confusion set	Reported symptom: subjective.
D	Adjacent category	A self-report even though it is a number.

3. Answer B Concept 2

A new patient, ~~a retired pilot who speaks three languages,~~ arrives at a clinic ~~with a newly remodeled waiting room~~ for a visit about fatigue. What is the best first question?

Target word: first interview question **Decides:** Open-ended to start.

A	Adjacent category	Leading question that can bias the answer.
B	Correct	Open-ended invites the patient's story.
C	Camouflage	Leads with his old job, an assumption.
D	Inverted operation	Closed question; belongs later.

4. Answer C Concept 3

A 30-year-old graduate student ~~with two cats~~ reports a headache. The clinician learns where it is and rates its severity at 7/10, then moves on to the physical exam. ~~Her blood pressure is 118/76.~~ What is the most important gap?

Target word: missing history **Decides:** Only location and severity collected.

A	Camouflage	A normal BP is not the gap.
B	Absolute language	Order is flexible; completeness is the issue.
C	Correct	A full symptom analysis needs all OLDCARTS elements.
D	Confusion set	The cats are not the symptom history gap.

5. Answer C Concept 4

A new rapid test is studied in a large urban hospital. Of 100 patients who have the disease, 85 test positive. Of 400 who do not, 380 test negative. ~~The test costs \$12 and takes 15 minutes.~~ What is its sensitivity?

Target word: sensitivity **Decides:** True positives / all with disease.

A	Confusion set	That is specificity, 380 / 400.
B	Wrong stem number	Divides 85 by all 500 patients.
C	Correct	85 / 100.
D	Inverted operation	The false-negative rate, 15 / 100.

6. Answer A Concept 4 • transfer

Using the same study (85 of 100 with disease test positive; 380 of 400 without disease test negative), what is the specificity?

Target word: specificity **Decides:** True negatives / all without disease.

A	Correct	380 / 400.
B	Wrong stem number	Divides 380 by all 500 patients.
C	Confusion set	That is sensitivity.
D	Inverted operation	The false-positive rate, 20 / 400.

7. Answer D Concept 5

At a community health fair ~~sponsored by a local bank~~, a nurse checks blood pressure for adults who have never been diagnosed with hypertension. ~~Free water bottles are handed out.~~ What level of prevention is this?

Target word: level **Decides:** Screening undiagnosed people.

A	Inverted operation	Tertiary manages known disease.
B	Adjacent category	Screening targets hidden disease: secondary.
C	Confusion set	Primary prevents onset, like vaccines.
D	Correct	Screening finds disease early.

8. Answer B Concept 5 • transfer

A patient recovering from a stroke begins physical therapy to regain walking. What level of prevention is this?

Target word: level for rehab **Decides:** Limiting damage from existing disease.

A	Inverted operation	Primary prevents disease before it begins.
B	Correct	Rehab reduces disability from existing disease.
C	Absolute language	Tertiary prevention applies exactly then.
D	Confusion set	Secondary is early detection.

9. Answer D Concept 6

A nurse assessing a rural county drives through its neighborhoods noting housing, grocery stores, and parks. She also downloads the county's census data. ~~The county fair is held every August.~~ Which source is primary data?

Target word: primary data source **Decides:** Collected firsthand by the assessor.

A	Camouflage	Not an assessment data source.
B	Confusion set	Collected by others: secondary.
C	Absolute language	Use does not change who collected the data.
D	Correct	She collected it herself.

10. Answer A Concept 6 · transfer

Which source is secondary data for a community assessment?

Target word: identify secondary **Decides:** Collected by others.

A	Correct	Collected by another agency for another purpose.
B	Adjacent category	Her own survey is primary.
C	Inverted operation	Collected firsthand: primary.
D	Confusion set	Firsthand observation: primary.

11. Answer B Concept 7

A Spanish-speaking patient arrives with her bilingual 12-year-old son, ~~a straight A student.~~ She has abdominal pain and questions about sexual history are needed. The clinic has a phone interpreter service. What is the best approach?

Target word: interpreter choice **Decides:** Use a trained medical interpreter.

A	Confusion set	Assumption instead of asking.
B	Correct	Trained interpreters protect accuracy and privacy.
C	Dropped element	Omits needed assessment.
D	Camouflage	Grades do not make a child an appropriate interpreter.

12. Answer C Concept 8

A 58-year-old man with a history of heartburn ~~who coaches youth soccer~~ reports upper abdominal burning with sweating and shortness of breath during exertion. What should be ruled out first?

Target word: priority on differential **Decides:** Exertional symptoms with sweating suggest cardiac ischemia.

A	Camouflage	His coaching does not explain exertional sweating and dyspnea.
B	Adjacent category	Anchors on history; a can't-miss cause comes first.
C	Correct	Life-threatening causes are ruled out first.
D	Absolute language	Overgeneralized and premature.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owled.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	1	
Wrong stem number	A figure copied from the prompt that is not the answer.	2	
Adjacent category	The right idea in the wrong bucket.	5	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	8	
Absolute language	Always, never, guarantee — where judgment applies.	4	
Confusion set	Every option is real; the test is exact role-matching.	10	
Camouflage	Answers a stem fact that was never the question.	6	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Subjective vs. objective data	Subjective data are what the patient reports (symptoms, history); objective data are what the clinician observes, measures, or tests (signs).	Filing a symptom the patient describes as objective because it was written in the chart.	"Patient states" vs. a measured or observed finding.
2	Open-ended then focused questions	Start the interview with open-ended questions to let the patient tell the story, then narrow with focused and closed questions.	Starting with a rapid list of yes/no questions.	The first question of an interview or a new complaint.
3	Symptom analysis (OLDCARTS)	A full symptom history covers onset, location, duration, character, aggravating/alleviating factors, radiation, timing, and severity.	Stopping after location and severity.	A chief complaint with pieces of the history missing.
4	Sensitivity vs. specificity	Sensitivity = true positives / all with disease (rules out when negative); specificity = true negatives / all without disease (rules in when positive).	Using the wrong denominator or swapping the two.	A 2x2 table or counts of people with and without disease.
5	Levels of prevention	Primary prevents onset (vaccines, counseling); secondary detects early (screening); tertiary limits damage from existing disease (rehab).	Calling screening primary prevention.	Does the person already have the disease, and is it known?
6	Primary vs. secondary community data	Primary data are collected firsthand by the assessor (windshield survey, interviews); secondary data were collected by others (census, health department reports).	Calling any data used in the assessment primary.	Who collected the data and for what purpose.
7	Culturally humble assessment	Use a trained medical interpreter, ask about the patient's own beliefs and practices, and avoid assumptions based on group identity.	Using a family member as interpreter or assuming beliefs from ethnicity.	A language barrier or cultural practice in the scenario.
8	Can't-miss diagnoses first	In a differential, rule out life-threatening conditions before settling on the most likely benign cause.	Anchoring on the most common or first-considered diagnosis.	A common complaint with a red-flag detail.

Course scope (public description): Advanced Health Assessment of Patients and Populations builds on prior physical health assessment knowledge and skills acquired during undergraduate studies by focusing on the advanced assessment of biopsychosocial and sociocultural contexts in patients and populations across the life span. This course emphasizes the use of a comprehensive health promotion, disease prevention, and health restoration model to address health concerns in patients and communities. Students will acquire advanced assessment knowledge and skills for clinical interviewing, focused history taking, critical diagnostic reasoning, and

clinical decision-making using a problem- focused framework that integrates authentic experiences with practical knowledge of health patterns in patients and communities. There are no prerequisites for this course.